GCP Compliance Area	Verification Item	Scenario Evidence / Finding	Status	Corrective Action Required	GCP Reference
Informed Consent	Verify informed consent was obtained before any study-specific procedures.	Participant 004 signed the IRB-approved ICF before screening procedures.	Verified	No corrective action required.	ICH GCP: Informed Consent
Investigator Oversight	Verify the PI provides adequate oversight of delegated trial activities.	PI reviews safety data and signs key eligibility assessments; delegation is documented.	Verified	No corrective action required.	ICH GCP: Investigator Responsibilities
Delegation & Training	Verify study tasks are delegated to qualified, trained staff.	A newly hired CRC is listed on the delegation log for study visits and has completed general GCP training, but has no documented protocol-specific training for IMT-201 (Finding 3).	Not Verified	Suspend the CRC from performing delegated study-visit tasks until protocol-specific IMT-201 training is completed and documented in the training record. PI to confirm that the delegation start date does not precede the training date and to sign the updated log. Review whether the CRC performed any study task before training; if so, document it as a deviation and assess the impact.	ICH GCP: Investigator Responsibilities
Protocol Compliance	Verify participants are treated according to the approved protocol.	Participant 007 completed the Week 12 assessment three days outside the protocol-defined visit window, so the visit was not conducted according to the approved protocol (Finding 4).	Not Verified	PI to assess and document the effect of the out-of-window Week 12 visit on participant safety, dosing interval, and data. Record the event as a protocol deviation and report it to the sponsor as required. Identify the cause (scheduling) and implement a visit-window tracker that shows each participant's target date and allowed window for all remaining visits, including Week 24.	ICH GCP: Compliance with Protocol
Eligibility	Verify eligibility criteria are confirmed and documented before randomization/treatment.	Participant 009 was randomized after eligibility review, but source does not show that one protocol-required exclusion criterion was checked. Eligibility was not fully documented before randomization (Finding 5).	Not Verified	PI to determine immediately whether the exclusion criterion was assessed. If supporting evidence exists, add it to source as a signed, dated late entry. If not, assess the criterion now and confirm whether the participant was eligible. Report the deviation to the sponsor (and IRB as required) and consult the sponsor on continued participation if eligibility cannot be confirmed. Introduce a criterion-by-criterion eligibility checklist with PI signature required before randomization.	ICH GCP: Investigator Responsibilities
Investigational Product	Verify IMT-201/placebo is stored, dispensed, and accounted for appropriately.	IMT-201/placebo is stored in a temperature-monitored, access-controlled area. However, one daily temperature entry from the prior week is missing and the missing reading has not been assessed (Finding 6).	Not Verified	Retrieve the monitoring device record for the missing day and document whether storage stayed within the required range. File a signed, dated note explaining the gap; do not backfill the log as if recorded on the day. If an excursion cannot be ruled out, quarantine the affected product and notify the sponsor before further dispensing. Assign backup staff for daily temperature checks.	ICH GCP: Investigational Product Management
Safety Reporting	Verify AEs/SAEs and other safety information are assessed, documented, and reported appropriately.	The SAE requiring hospitalization was documented in source and reported to the sponsor within the required timeframe. A mild injection-site reaction for another participant is documented in source but has not been entered on the EDC AE page (Finding 7).	Not Verified	Enter the injection-site reaction on the EDC AE page, with the investigator's assessment of severity and relationship to study treatment documented in source. Reconcile source against EDC for all enrolled participants to find any other unentered AEs. Reinforce the data-entry timeline for AEs with all delegated staff. No action is required for the SAE.	ICH GCP: Safety Reporting

GCP Compliance Area	Verification Item	Scenario Evidence / Finding	Status	Corrective Action Required	GCP Reference
Source Documentation	Verify source records are attributable, contemporaneous, accurate, and support reported trial data.	Most source notes are signed and dated. One Week 8 visit note contains a late-added clinical clarification that does not identify who made the addition or when (Finding 8).	Not Verified	Identify the author of the addition. The author must initial or sign and date it with the actual date of entry, mark it as a late entry, and state the reason, without obscuring the original note. If the author cannot be identified, document that in a signed note to file. Retrain staff on attributable, contemporaneous documentation and on making corrections that preserve the original entry.	ICH GCP: Records and Reports
Essential Records	Verify required regulatory and essential records are current and available.	The regulatory binder contains the current protocol, IRB approval, investigator documents, and training records. The most recent laboratory certification has expired and the renewed certificate is not filed (Finding 9).	Not Verified	Obtain the current laboratory certificate and file it in the regulatory binder. Confirm that certification was continuous for the period in which study samples were analyzed; if there was a lapse, notify the sponsor and assess the affected results. Add expiry dates for all time-limited essential records to a tracker that is reviewed regularly.	ICH GCP: Essential Records
Privacy & Confidentiality	Verify participant-identifying information is protected and access is limited.	Participant files are kept in access-controlled storage. A screening worksheet showing a participant name and study ID was found left on an open desk in a shared work area (Finding 10).	Not Verified	Secure the worksheet in the participant file immediately. Assess whether anyone without authorization could have viewed it and report the event under the institution's privacy incident procedure and to the PI. Retrain staff on confidentiality and require that documents with participant identifiers are never left unattended in shared areas.	ICH GCP: Confidentiality
Laboratory Procedures	Verify protocol-required laboratory testing is performed and clinically significant abnormalities are followed.	Protocol-required laboratory tests are present for Participant 011. A clinically significant abnormal result was flagged by the laboratory, but source contains no PI or sub-investigator assessment or follow-up (Finding 11).	Not Verified	PI or sub-investigator to review the result promptly and, before the participant's next injection, sign and date the report, document clinical significance, and record the follow-up plan. Record an adverse event if the investigator judges it applicable and report it per protocol. Review the laboratory reports of all participants for other unassessed results and require investigator sign-off on every report on receipt.	ICH GCP: Medical Care / Safety
Protocol Deviations	Verify deviations are documented, evaluated, reported when required, and corrective action is implemented.	The site maintains a deviation log, but the Week 12 out-of-window visit for Participant 007 has not been documented or evaluated for reporting requirements (Finding 12).	Not Verified	Enter the Participant 007 deviation on the log with date, description, PI assessment, and reporting decision, and report it to the sponsor and IRB as their requirements specify. Log the other deviations identified in this review (Participant 009 eligibility documentation; delegation before protocol training). Review all completed visits against protocol windows for further unlogged deviations, and have the PI review the log on a regular schedule.	ICH GCP: Compliance with Protocol

Phase II Immunotherapy Trial — Simulated Site Qualification Visit Scenario

Study Context

The site is preparing for a sponsor site qualification/readiness assessment for a randomized, controlled Phase II trial of IMT-201 versus placebo. The primary objective is to evaluate reduction in disease activity from baseline to Week 24. Secondary objectives include physical function, participant-reported pain, and safety/tolerability, including AEs, SAEs, injection-site reactions, clinically significant laboratory abnormalities, and treatment discontinuations.

Simulated Site Visit Findings

1. Participant 004 signed the current IRB-approved informed consent form before any screening or other study-specific procedures were performed.

2. The PI reviews safety information and signs key eligibility assessments. Staff responsibilities are recorded on the delegation log.

3. A newly hired CRC completed general GCP training but has not yet completed documented protocol-specific training for IMT-201. The CRC is listed on the delegation log for study visits.

4. Participant 007 completed a Week 12 assessment three days outside the protocol-defined visit window. The event has not yet been entered on the site's deviation log.

5. Participant 009 was randomized after eligibility review, but the source file does not contain documentation showing that one protocol-required exclusion criterion was checked.

6. IMT-201/placebo is stored in a temperature-monitored, access-controlled area. During review, one daily temperature entry from the prior week is missing and there is no documented assessment of the missing reading.

7. An SAE requiring hospitalization was documented in source and reported to the sponsor within the required timeframe. A mild injection-site reaction for another participant is documented in source but has not yet been entered in the EDC AE page.

8. Most source notes are signed and dated. One Week 8 visit note contains a late-added clinical clarification without identifying the person who made the addition or the date it was added.

9. The regulatory binder contains the current protocol, IRB approval, investigator documents, and training records; however, the most recent laboratory certification has expired and the renewed certificate is not filed.

10. Participant files are kept in access-controlled storage. A screening worksheet containing participant name and study ID was found temporarily left on an open desk in a shared work area.

11. Protocol-required laboratory tests are present for Participant 011. A clinically significant abnormal result is flagged by the laboratory, but the source does not document PI/sub-investigator assessment or follow-up.

12. The site maintains a deviation log, but the Week 12 out-of-window visit noted above has not been documented or evaluated for reporting requirements.

Learner Instructions

Use the scenario above to complete every remaining row of the GCP Compliance Checklist. For each item, select Verified, Not Verified, or Not Applicable. When a gap exists, summarize the finding and document a specific corrective action. Two checklist items are completed as models.